



Wild yam is found growing in Mexico. Its rhizome relaxes smooth muscle and it is used as an antispasmodic.

to harmonize “the organic tissues with the vital force,” with the aim of restoring equilibrium within the body. Believing the stomach was the source of disease, Physiomedicalists used herbs that induced vomiting, such as pokeweed (*Phytolacca americana*, p. 247), to cleanse the organ. Other herbs, such as echinacea (*Echinacea* spp., p. 92), now recognized as an effective immune modulator,

and goldenseal (*Hydrastis canadensis*, p. 105), a tonic and anti-inflammatory, were then prescribed to aid recovery.

The second half of the 19th century was an extraordinary time for American natural medicine. In addition to engendering osteopathy and chiropractic at the turn of the century, it also reinvigorated herbal medicine in Britain to such a degree that Physiomedicalism became an Anglo-American herbal tradition. To this day, British herbalists still use a far wider variety of North American medicinal herbs than do their European counterparts.

North American Herbal Medicine Today

In the U.S., herbal medicine went into steep decline after 1907 because of the government’s decision to limit financial support for herbal medical training at conventional medical schools. Since that time, herbal medicine in both the U.S. and Canada has existed mostly on the fringes of conventional health care. In some U.S. states, it is illegal to practice herbal medicine without medical qualifications, but courses in herbalism are not offered at medical schools.

With the passing of liberal legislation in 1994, use of herbal medicine in the U.S. exploded and sales of St. John’s wort (*Hypericum perforatum*, p. 106)—the most extreme example—are thought to have increased by 3,900 percent between 1995 and 1997. Such growth could not be sustained, and led to lower-quality herbal products being sold over the counter. The American Botanical Council has been at the forefront of the campaign to raise quality standards of herbal medicines, and many bona fide herbal manufacturers have signed up to their Botanical Adulterants program. At the same time, greater resources are being put into researching herbal medicines, for example through the U.S. National Center for Complementary and Integrative Health, which has so far funded over 15 clinical trials into herbs such as garlic (*Allium sativum*, p. 59) and saw palmetto (*Serenoa repens*, p. 136).

Over the past 20 years, North Americans have become increasingly savvy about herbal medicines and their role in maintaining well-being and treating ill health. Best-selling herbs include turmeric (*Curcuma longa*, p. 90) and elderberry (*Sambucus nigra*, p. 132), and their use fits well with the American tradition of self-reliance.



Turmeric

Damiana (*Turnera diffusa*, p. 144) eases depression.

Cayenne (*Capsicum frutescens*, p. 74) is a potent warming herb that stimulates circulation and digestion.

Evening primrose (*Oenothera biennis*, p. 241) yields a seed oil containing essential fatty acids that help maintain healthy tissues.

Black cohosh (*Cimicifuga racemosa*, p. 79) was used by Native Americans to treat rheumatism.

Helonias (*Chamaelirium luteum*, p. 186) was chewed by Native American women to prevent miscarriage.